

Surgery for colon cancer

Surgery is the most common treatment for colon cancer. The type of surgery you have will depend on the size of the cancer, where it is in the colon and whether it has spread.

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Colon cancer surgery

Surgery is the most common treatment for [colon cancer](#). The type of surgery you have will depend on:

- the [size of the cancer](#)
- where it is in the [colon](#)
- whether it has spread.

Your doctor will discuss this with you.

There are things you can do [before](#) and [after surgery](#) to help you prepare and recover.

[Bowel cancer](#) is a general term for cancer that starts in either the colon or rectum. It might sometimes be called colorectal cancer.

We have separate information about [surgery to the rectum](#). The types of operation used for rectal cancer are different to the types of operation used for colon cancer.

In the video below, Oncologist Rebecca and Cancer Nurse Specialist Claire explain how bowel cancer may be treated. They talk about surgery, chemotherapy, radiotherapy, immunotherapy and targeted therapies.

[YouTube video player](#)

We understand that having treatment can be a difficult time for people. We are here to support you. If you want to talk, you can:

- call the Macmillan Support Line for free on [0808 808 00 00](#)
- [chat to our specialists online](#)
- visit our [bowel \(colon and rectal\) cancer forum](#) to talk to people who have been affected by bowel cancer, share your experience, and ask an expert your questions.

Before your operation

Before your operation, you go to a pre-assessment clinic to check you are fit for the operation. At the clinic, you may have [blood tests](#), a blood pressure check and a [recording of your heart \(ECG\)](#).

For some types of colon cancer surgery, your bowel needs to be empty. A nurse may advise you about preparing for your operation. You may need to follow a special diet. You may need to take a laxative for a few days before the operation. You will also be asked not to eat or drink for a few hours before the operation.

Before the operation, you will usually be given antibiotics. These are to prevent infection. You may have them as an injection or tablets.

You will meet a member of the surgical team to discuss the operation. If you are going to have a [stoma](#), you will also meet a stoma care nurse, who will explain what is involved.

During your clinic appointment, you can ask questions or share any concerns you have about the operation. If you live alone, or care for someone else, you may need help when you go home after surgery. Tell a nurse as soon as possible. They can help to make arrangements for you. We have more information about [getting support at home after leaving hospital](#).

You will usually go into hospital on the morning of the operation. You will be given elastic stockings (TED stockings) to wear during the operation and for some time afterwards. This is to prevent [blood clots](#) in your legs.

Removing very small, early-stage colon cancers

Some very small, early-stage colon cancers can be removed from the bowel lining using a [colonoscope](#). This means the surgeon does not have to make a cut in your tummy (abdomen). The operation is sometimes called a local resection.

For a local resection, you may have a general anaesthetic. Or you will be given drugs called sedatives to make you sleepy. The surgeon passes a colonoscope into your back passage (rectum) and along the bowel until they reach the cancer. They use small cutting tools to remove the cancer and some healthy tissue (margins) from the affected area.

A doctor called a pathologist examines the cancer that has been removed to find out its [grade](#).

Your surgeon may recommend a second operation to remove more of the colon if:

- the cancer cells look very abnormal (high-grade)
- the cancer has not been completely removed.

Removing part or all of the colon

Surgery to remove part or all of the colon is called a resection or colectomy. How much of the colon is removed depends on the size of the cancer and where the cancer is.

After the surgeon has removed the affected part of the colon, they join the 2 open ends of the bowel together. Sometimes they may need to create a [stoma](#). The surgeon will usually remove the [lymph nodes close to the colon](#). These are tested for cancer cells in a laboratory.

There are different types of colectomy:

- Total colectomy – the surgeon removes the whole colon. They bring the end of the small bowel (ileum) to the surface of the tummy to make a stoma. This is called an ileostomy. Sometimes the surgeon is able to join the small bowel to the rectum, so an ileostomy is not needed.
- Left hemi-colectomy – the surgeon removes the left half of the transverse colon and the descending colon.
- Right hemi-colectomy – the surgeon removes the right half of the transverse colon and the ascending colon.

These diagrams show the parts of the colon removed during these operations.

Left hemi-colectomy

Image: The diagram shows the position of the colon in the abdomen. The colon is shown in orange. Part of the left half of the transverse colon and the descending colon is shown in white. This part of the colon is removed during a left hemi-colectomy.

Right hemi-colectomy

Image: The diagram shows the position of the colon in the abdomen. The colon is shown in orange. Part of the right half of the transverse colon and the ascending colon is shown in white. This part of the colon is removed during a right hemi-colectomy.

Some people have an operation to remove the sigmoid colon (the S-shaped bend that joins the colon to the rectum). This is called a sigmoid colectomy or high anterior resection. The surgeon may join the 2 open ends of bowel together. Or they may need to create a type of stoma called a colostomy.

Sigmoid colectomy

Image: The diagram shows the position of the colon in the abdomen. The colon is shown in orange. The sigmoid colon, which is bottom left, is shown in white. This part of the colon is removed during a sigmoid colectomy.

Some people have an operation to remove the transverse colon (the top part of the colon). This is called a transverse colectomy.

Transverse colectomy

Image: The diagram shows the position of the colon in the abdomen. The colon is shown in orange. The transverse colon is the part that goes across the top of the abdomen. This is shown in white. This part of the colon is removed during a transverse colectomy.

Open or laparoscopic surgery

Your operation may be done as open surgery or keyhole surgery. This is called laparoscopic surgery.

Open surgery means the surgeon makes 1 large cut, usually near the tummy button. The length of the cut depends on the size of the cancer and where it is.

Laparoscopic surgery means the surgeon makes 4 or 5 small cuts in the tummy. They pass a thin tube through 1 of the cuts. The tube is called a laparoscope. It has a small camera on the end. The surgeon then passes specially designed surgical tools through the other cuts. The surgeon uses these tools to remove the cancer.

Laparoscopic surgery is also sometimes called minimally invasive surgery. You usually recover more quickly from it than from open surgery.

Robotic surgery

Robotic surgery is when laparoscopic surgery is helped by a machine. The laparoscope and the surgical equipment are attached to robotic arms. The surgeon controls the robotic arms, which can move very precisely. This is very specialised surgery and only available in some hospitals in the UK.

Your surgeon will talk to you about which type of surgery is likely to be best in your situation.

Stomas (colostomy or ileostomy)

Sometimes during surgery the surgeon needs to create a stoma. This is an opening the surgeon makes through the tummy (abdominal) wall. It connects the bowel to the surface of the tummy. The stoma is round or oval-shaped, and it looks pink and moist. It has no nerve supply, so it does not hurt.

Having a stoma means stools (poo) and wind will not pass out of the rectum and anus in the usual way. Instead, they will pass out of the stoma, into a disposable bag you wear over the stoma.

If the stoma is made from an opening in the colon, it is called a colostomy. If it is made from an opening in the small bowel (ileum), it is called an ileostomy.

Stomas may be temporary or permanent. Only a small number of people with colon cancer need a permanent stoma.

The surgeon may make a temporary stoma to allow the bowel to heal after surgery. If you have a temporary stoma, you will usually have an operation to close the stoma and rejoin the bowel after you have finished your treatment. This operation is called a stoma reversal.

Your surgeon will tell you whether you are likely to have a stoma, and whether it will be temporary or permanent. If you need to have a stoma, you will be referred to a nurse who specialises in stoma care. They can talk to you about what to expect.

We have more information about [stomas](#).

Surgery for advanced colon cancer

If the cancer has spread to other parts of the body, you may be able to have surgery to remove it. It can sometimes lead to a cure.

Surgery is only suitable for some people with [secondary liver cancer](#) or [secondary lung cancer](#). You can talk to your cancer doctor about whether this surgery may be helpful for you. Some people might need surgery if the cancer is causing a [blockage in the bowel](#).

We have more information about [treatment for colon cancer that has spread to other parts of the body](#).

After your operation

Your nurse will explain what to expect after your operation. They will help you to get out of bed or sit up for a short time as soon as you are able to.

You are likely to have a drip in your arm to give you fluids until you are eating and drinking again. You may have a tube in your tummy, close to the wound, to drain fluid into a drainage bag or bottle. Your nurse will explain any other tubes.

We have more information on [recovering after bowel cancer surgery](#).

Booklets and resources



Booklets

30 May 2024

Understanding colon cancer

About our information

This information has been written, revised and edited by Macmillan Cancer Support's Cancer Information Development team. It has been reviewed by expert medical and health professionals and people living with cancer.

References



Below is a sample of the sources used in our colon cancer information. If you would like more information about the sources we use, please contact us at informationproductionteam@macmillan.org.uk

National Institute for Health and Care Excellent (NICE). Colorectal cancer: diagnosis and management clinical guidelines. Updated December 2021. Available from: www.nice.org.uk/guidance/NG151 [accessed November 2023].

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Cervantes A, Adam R, Rosello S, et al. Metastatic colorectal cancer: ESMO Clinical Practice Guideline for diagnosis, treatment and follow-up. Annals of Oncology. 2023 Jan; 34(1):10-32. doi: 10.1016/j.annonc.2022.10.003. Epub 2022 Oct 25. Available from: [www.annalsofoncology.org/article/S0923-7534\(22\)04192-8/fulltext](http://www.annalsofoncology.org/article/S0923-7534(22)04192-8/fulltext) [accessed February 2023].

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Our cancer information meets the [PIF TICK](#) quality mark.

This means it is easy to use, up-to-date and based on the latest evidence. Learn more about [how we produce our information](#).

The language we use

We want everyone affected by cancer to feel our information is written for them.

We want our information to be as clear as possible. To do this, we try to:

- use plain English
- explain medical words
- use short sentences
- use illustrations to explain text
- structure the information clearly
- make sure important points are clear.

We use gender-inclusive language and talk to our readers as 'you' so that everyone feels included. Where clinically necessary we use the terms 'men' and 'women' or 'male' and 'female'. For example, we do so when talking about parts of the body or mentioning statistics or research about who is affected.

You can read more about how we produce our information [here](#).

How we can help

Clinical Information Nurse Specialists

Our Cancer Information Nurse Specialists are dedicated cancer nurses available to talk to on our Macmillan Cancer Support Line.



Call us free on 0808 808 0000

8am to 8pm. 7 days a week. You can talk to our cancer specialists about anything.

Email the Macmillan Support Line



Email the Macmillan Support Team if you have any questions.

Chat online

Available 8am to 7.45pm, 7 days a week. For confidential, real-time support if you don't want to call.



Bowel (colon and rectal) cancer forum

An anonymous network of people affected by colon, rectal, small bowel, or other colorectal cancers, which is free to join. Share experiences, ask questions and talk to people who understand.



Find cancer support services in your area

What's going on near you? Find out about support groups, where to get information and how to get involved with Macmillan where you live.

Get help